AFFIDAVIT

STATE OF ARIZONA

CITY OF GILBERT

I, KEVIN TURNAGE make the following statement to Jacqlynn Klein, who has identified herself as an EEO Contract Investigator for the Department of Health and Human Services, Indian Health Service ("IHS"). I understand this statement is not confidential and may be shown to the interested parties (those with a legal right to know). I understand that this statement may be used in evidence.

I understand that the Indian Health Service has accepted for investigation the following claims of Tonie Greve (Complainant):

Complainant alleged she was subjected to discrimination based on race (American Indian), color (dark), age (Y.O.B. 1965), national origin (member of Omaha Tribe of Nebraska), disability (physical/mental), reprisal (opposing discrimination) and subjected to harassment (non- sexual)/hostile work environment when:

1. Complainant alleged she was subjected to harassment/hostile work environment. Examples include, but not limited to:

    a. Demeaning comments made by Complainant's Supervisor, Kevin Turnage, which include, "the United States government does not care for its veterans, and it certainly does not care for American Indians." "I will fill all jobs with non-Indians." Complainant further alleged Turner looks at her dark skin and forgets that she is an enrolled American Indian, not African American, and feels free to make derogatory comments in her presence; and,
    b. Complainant alleged Phoenix Indian Medical Center (PIMC) does not advertise jobs using Indian Preference, so that Complainant can apply.

2. Complainant alleged she was assigned eight (8) doctors while co-workers Elizabeth Hill and Bertha Edison (both members of tribes recognized in Arizona) were assigned five (5) and four (4) doctors, respectively;
3. Complainant alleged she was forced to cover Clinical RN duties while others are not. Further, Complainant alleged Clinical RN Nurse Assessment/Nurse Triage were not part of her official duties. Complainant alleged Brenda Barr and Arlene Frost (both Caucasian nurses) were assigned these duties. Complainant also alleged she was required to perform the duties of Medical Assistants and Medical Staff Assistants;
4. Complainant alleged the abovementioned duties were not included in her PMAP and that she was rated the same as Caucasian employees with less duties;
5. Complainant alleged she did not have sick leave put back into the E-Comp leave system while other employees did; and,

6. Complainant alleged she was told she needed to provide a doctor's note for her immunocompromised status and reasons that she should not be placed on front line RN triage. Complainant further alleged other Caucasian and contract employees were not required to submit this documentation.

On January 17, 2023, Complainant requested an amendment to her formal complaint.

Complainant alleged she was subjected to discrimination based on race (American Indian), color (dark), age (Y.O.B. 1965), national origin (member of Omaha Tribe of Nebraska), disability (physical/mental), reprisal (opposing discrimination) and subjected to harassment (non- sexual)/hostile work environment when:

7. Complainant alleged her reasonable accommodation, which she says was on file since February 21, 2022, was removed. Complainant further alleged she was being required to repeatedly submit forms to put an accommodation back in place; and,
8. Complainant alleged she was being required to swab symptomatic respiratory patients which was not part of her official duties.

Q1. What state and county are you located in as you answer these questions.

A1. Arizona Maricopa County

Q2. Please state your full name for the record.

A2. Kevin Dean Turnage

Q3. Please state your pronouns. (Ex: he/him/his, she/her/hers, they/them/their)

A3:

Q4. For what Agency do you work?

A4: Indian Health Services

Q5. What is your current title, series and grade, to what organizational unit/department are you assigned, and what is your duty location (city/state)?

A5: Policy Officer (ACTING) GS 0610 12 09 Phoenix Indian Medical Center/Nursing/quality/teleworking

Q6. For how long have you been in your current position? (If you were in a different position at the time of the alleged discrimination, please identify the position you held at that time by title, series, grade, organizational unit/department, and duty location.)

A6. Since April 2023.

Q7. Please identify your current first- and second-line supervisors, including name and title.

A7.

First-Line: Endryea McCabe, Director of Quality and Performance Improvement

Second Line: Clinton Pope, Chief Quality Officer

Q8. What was your job title, series/grade, organizational unit/department you were assigned, and what was your duty location (city/state) during the time of allegations: July 19, 2022, and August 12, 2022?

A8: Phoenix Indian Medical Center/Nursing/Primary Care/ Supervisory Clinical Nurse, Phoenix Arizona

Q9. Please identify your first- and second-line supervisors, including name and title, as of July 19, 2022, and August 12, 2022

A9:

First-Line: Jason White/Brandi Benjamin (Acting) Ambulatory Chief Nursing Officer/ Ambulatory Chief Nursing Officer

Second Line: Candace Lee, Chief Nursing Officer

Q10. As this complaint has been filed based on race (American Indian), color (dark), age (Y.O.B. 1965), national origin (member of Omaha Tribe of Nebraska), disability (physical/mental), reprisal (opposing discrimination).

A. Please identify your race, color, age (month/year), and national origin for the record.

B. Have you identified yourself to the Agency as an individual with a disability?

C. Have you engaged in prior EEO activity (i.e., formal or informal EEO complaint; witness in investigation of coworker's complaint; request for reasonable accommodation; verbal complaint about discrimination against oneself or others; threatening to file a charge of discrimination; etc.)?

A10:

A. Caucasian Male 50 (04/1973) American

B. Yes

C. Yes

Q11. What was your organizational relationship (if any) to Tonie Greve as of July 19, 2022, and August 12, 2022?

A11. Supervisor

Claim 1: Complainant alleged she was subjected to discrimination based on race (American Indian), color (dark), age (Y.O.B. 1965), national origin (member of Omaha Tribe of Nebraska), disability (physical/mental), reprisal (opposing discrimination) and subjected to harassment (non- sexual)/hostile work environment when:

Complainant alleged she was subjected to harassment/hostile work environment. Examples include, but not limited to:

a. **Demeaning comments made by Complainant's Supervisor, Kevin Turnage, which include, "the United States government does not care for its veterans, and it certainly does not care for American Indians." "I will fill all jobs with non-Indians." Complainant further alleged Turner looks at her dark skin and forgets that she is an enrolled American Indian, not African American, and feels free to make derogatory comments in her presence; and,**
b. **Complainant alleged Phoenix Indian Medical Center (PIMC) does not advertise jobs using Indian Preference, so that Complainant can apply.**

Q12. For the alleged discrimination:

"Demeaning comments made by Complainant's Supervisor, Kevin Turnage, which include, "the United States government does not care for its veterans, and it certainly does not care for American Indians." "I will fill all jobs with non-Indians." Complainant further alleged Turner looks at her dark skin and forgets that she is an enrolled American Indian, not African American, and feels free to make derogatory comments in her presence; and,"

Please explain in detail when this happened (to include dates), where this incident happened, who all was involved in this incident, and what transpired? Be sure to include the nature of your involvement.

A12. Specifically Denied as nothing happened, this statement is slanderous, fabricated defaming, untruthful and if made under oath it is perjury as this never occurred and is completely fabricated.

Q13. Do you feel the incidents you described above, taken as a whole, constituted harassment and a hostile work environment for the Complainant?

A13. No fabricated events cannot cause harassment or a hostile work environment.

Q14. Are you aware of any Agency policy that guides employees about what to do if/when they feel they are being subjected to harassment in their workplace? If so, briefly describe your understanding of what an employee should do in such a situation.

A14. Yes, notify the chain of command

Q15. Was the Complainant's alleged harassment reported to the Complainant's chain of command? If so, to whom did the Complainant make the report to, on what date, what information did the Complainant share about the nature of her concerns, and what was the response to the report of alleged harassment?

A15. I have insufficient information to respond to this question as the complaint never informed me of her concerns or that she felt she was being harassed. I know she complained to someone but I am not aware of whom or when.

Q16. If the Complainant reported the alleged harassment to anyone in their chain of command, did they take any actions/measures to address the concerns the Complainant had raised? If so, what actions were taken, when and by whom?

A16. Not Applicable since I have no knowledge if she notified anyone above me.

Q17. How does the Phoenix Indian Medical Center (PIMC) advertise jobs when it comes to race or national origin? What are their hiring procedures when it comes to race or national origin?

A17. I cannot completely answer this question as after a request to fill a position is completed, it is sent to Human Resources, which has several different procedures before posting. I am aware of Federal Law regarding Indian Preference (IP) in which American Indian/Alaska Native applicants receive preference over non-Indian applicants when applying for jobs with the Indian Health Service (IHS) and the Bureau of Indian Affairs (BIA). They are eligible for preference if they (a) are a member of a federally recognized Indian tribe; (b) are a descendent of a member and were residing within the present boundaries of any Indian reservation on June 1, 1934; (c) are an Alaska Native; or (d) possess one-half degree Indian blood derived from tribes that are indigenous to the United States. After posting closes a panel of qualified candidates is sent to the hiring manager for interviews.

If there are qualified IP candidates for the position from applicants, then the hiring manager receives only those candidates, and if none of the IP candidates are chosen, then the position is blocked from being filled for 180 days at which time the position can be reposted. Human Resources handles all posting and management is only involved in approving the job description and then interviewing we have no control of any other aspect of the process. I can refer you to the PIMC policy *"New Employee Selection Guidelines"* and the "*Indian Health Manual Chapter Three part seven"* which covers Human Resource Administration and Management. Additional guidance is received from the Department of Health and Human Services. All jobs are posted on USA as far asjobs.gov. The rest of the process for advertisement is dictated and controlled by human resources.

Claim 2: Complainant alleged she was subjected to discrimination based on race (American Indian), color (dark), age (Y.O.B. 1965), national origin (member of Omaha Tribe of Nebraska), disability (physical/mental), reprisal (opposing discrimination) and subjected to harassment (non- sexual)/hostile work environment when:

Complainant alleged she was subjected to harassment/hostile work environment. Examples include, but not limited to: Complainant alleged she was assigned eight (8) doctors while co-workers Elizabeth Hill and Bertha Edison (both members of tribes recognized in Arizona) were assigned five (5) and four (4) doctors, respectively;

Q18. Do you believe age was a factor in this action?

A18. No

Q19. Do you believe color was a factor in this action?

A19. No

Q20. Do you believe race and national origin was a factor in this action?

A20. No

Q21. Do you believe disability (mental/physical) was a factor in this action?

A21. NO

Q22. Do you believe reprisal (opposing discrimination) was a factor in this action?

A22. No

Q23. Was anyone treated more favorably than the Complainant in a similar circumstance? Identify by name, bases of the complaint, including specific age, circumstances, when the treatment occurred, management involved.

A23. No

Q24. For this alleged discrimination, please identify: what happened, when it happened, and who made it happened, and the nature of your involvement. Please be very detailed.

A24. Nothing happened

Q25. What is the Phoenix Indian Medical Center (PIMC) policy for nurse assignments to doctors?

A25. Clinical Care Coordinator (CCC), the position held by the complainant, is not assigned to doctors but teams. They are to work as part of the interdisciplinary team to ensure a patient receives the care ordered. I refer to the complainant's official position description *PX3439 PCMC CCC (02.28.21)*. These assignments were the assignments in place when I accepted the position. When I did attempt to change assignments, the complainant complained because I was changing assignments whether it was to ensure patient panel size or if it was related to the number of doctors assigned. The CCCs were all allowed to provide input into the changes. There's insufficient information to answer this question, as assignments vary depending on inpatient, outpatient, specialty, staffing, and other factors. Email chain provided with ccc input on the division of work.

Claim 3: Complainant alleged she was subjected to discrimination based on race (American Indian), color (dark), age (Y.O.B. 1965), national origin (member of Omaha Tribe of Nebraska), disability (physical/mental), reprisal (opposing discrimination) and subjected to harassment (non- sexual)/hostile work environment when:

Complainant alleged she was subjected to harassment/hostile work environment. Examples include, but not limited to: Complainant alleged she was forced to cover Clinical RN duties while others are not. Further, Complainant alleged Clinical RN Nurse Assessment/Nurse Triage were not part of her official duties. Complainant alleged Brenda Barr and Arlene Frost (both Caucasian nurses) were assigned these duties. Complainant also alleged she was required to perform the duties of Medical Assistants and Medical Staff Assistants;

Q26. For this alleged discrimination, please identify: what happened, when it happened, and who made it happened, and the nature of your involvement. Please be very detailed.

A26. I have no knowledge of her being subjected to a hostile work environment. Four other nurses claim that the complainant creates a hostile work environment.

Q27. Do you believe age was a factor in this action?

A27. No

Q28. Do you believe color was a factor in this action?

A28. No

Q29. Do you believe race and national origin was a factor in this action?

A29. No

Q30. Do you believe disability (mental/physical) was a factor in this action?

A30. No

Q31. Do you believe reprisal (opposing discrimination) was a factor in this action?

A31. No

Q32. What knowledge do you have of the Complainant's alleged harassment where she was forced to cover Clinical RN duties? Did she question why she was forced to

cover these duties? Please explain why these duties were assigned to the Complainant.

A32. 75% of her job is described as clinical duties. She was never forced she was asked to complete an assignment that is in line with and included in her official position description *PX3439 PCMC CCC (02.28.21)*, which states:

Recognizes urgent/emergent patient and clinic situations and initiate emergency medical procedures…(which by definition is triage) …Uses appropriate medical supplies and/or equipment for the assigned clinic, such as 12-lead EKGs, fetal monitor, pulse oximeter, etc. and collects patient specimens, including blood draw, if needed. Processes lab specimens for transport or perform tests, such as waived tests, to obtain results. Perform other related duties as assigned.

All CCC's are expected to cover the Nurse Assessment (Triage) Room when the nurse assigned to the room is out. They were assigned in rotation.

Q33. Did the Complainant report to anyone in their chain of command that she felt the actions/treatment at issue in this complaint were harassing and/or were creating a hostile work environment for her? If so, to whom did she make her report to, on what date, what information did she share with you about the nature of her concerns, and how did you respond?

A33. Not that I am aware of.

Q34. If the Complainant reported the alleged harassment to anyone in her chain of command, did management take any actions/measures to address the concerns she had raised? If so, what actions were taken, when, and by whom?

A34. Not Applicable

*Q35. Please describe in detail the Complainant's job duties as a Clinical Care Coordinator, RN, GS-610, 10-7*A32.

A35. I refer to her official position description *PX3439 PCMC CCC (02.28.21)* 75% Clinical Care Duties 25% Coordination Lead Duties. This was also provided numerous times to the complainant, and she refused to discuss how the duty assignments she questioned were in alignment with the noted position description.

Claim 4: Complainant alleged she was subjected to discrimination based on race (American Indian), color (dark), age (Y.O.B. 1965), national origin (member of Omaha Tribe of Nebraska), disability (physical/mental), reprisal (opposing discrimination) and subjected to harassment (non- sexual)/hostile work environment when:

Complainant alleged she was subjected to harassment/hostile work environment. Examples include, but not limited to: Complainant alleged the abovementioned duties were not included in her PMAP and that she was rated the same as Caucasian employees with less duties;

Q36. For this alleged discrimination, please identify: what happened, when it happened, and who made it happened, and the nature of your involvement. Please be very detailed.

A36. I am unaware of discrimination based on race, color, age, national origin, disability, or reprisal. Since I do not have knowledge of her rating or score there is insufficient information for me to respond to this question

Q36. Do you believe age was a factor in this action?

A36. No

Q37. Do you believe color was a factor in this action?

A37. No

Q38. Do you believe race and national origin was a factor in this action?

A38. No

Q39. Why do you believe disability (mental/physical) was a factor in this action?

A39. I do not believe it was.

Q40. Do you believe reprisal (opposing discrimination) was a factor in this action?

A40. No

Q41. What duties were included in the Complainant's PMAP? What ratings did she received? Were her ratings abnormal from her previous ratings?

A41. I did not complete the employee's PMAP it was completed by Jason White. I do not know what it contains, how it was scored, or the final score. Since I do not have knowledge of her rating or score, there is insufficient information for me to respond to this question.

Q42. Did Caucasian employees with less duties job titles receive the same score as the Complainant? Who were these employees? Where do the employees view the PMAP ratings and how do they know if they received the same scores as another employee?

A42. All CCCs have the same job duties, so I do not know whom she refers to. I am not at liberty to discuss the scores of other employees without their consent.

Q43. Please describe what PMAP is and the agency policy for rating employees.

A43. Performance Management and Appraisal Program (PMAP) ratings are described in accordance with:

5 U.S.C. Chapter 43, Performance Appraisal, and 5 CFR Part 430, Performance Management

5 U.S.C. §§4303 and 4305 and 5 CFR Part 432.104, Achieved Unsatisfactory Results (UR) Performance

5 U.S.C. Chapter 45 and 5 CFR Part 451, Awards

5 U.S.C. §§5335 and 5304 and 5 CFR Part 531, Subpart D, Within-Grade Increases

5 U.S.C. §5336 and 5 CFR Part 531, Subpart E, Quality Step Increases

5 U.S.C. §3502 and 5 CFR §351.504, Reduction-in-Force

5 U.S.C. §§552a and 5 CFR §293.404 and 5 CFR §293.405, Employee Performance Records

5 CFR §432.104, Addressing Unacceptable Performance

Executive Order 13839, Promoting Accountability and Streamlining Removal Procedures Consistent with Merit System Principles, May 25, 2018

OMB Memorandum, Comprehensive Plan for Reforming the Federal Government and Reducing the Federal Civilian Workforce (M-17-22), April 17, 2017

HHS Performance Management Appraisal Program Handbook, 2016

All applicable HHS Collective Bargaining Agreements

Previously dictated by the Indian Health Manual Chapter 7

Claim 5: Complainant alleged she was subjected to discrimination based on race (American Indian), color (dark), age (Y.O.B. 1965), national origin (member of Omaha Tribe of Nebraska), disability (physical/mental), reprisal (opposing discrimination) and subjected to harassment (non- sexual)/hostile work environment when:

Complainant alleged she was subjected to harassment/hostile work environment. Examples include, but not limited to: Complainant alleged she did not have sick leave put back into the E-Comp leave system while other employees did; and,

Q44. For this alleged discrimination, please identify: what happened, when it happened, and who made it happened, and the nature of your involvement. Please be very detailed.

A44. The complainant was informed numerous times to contact the Department of Labor as COVID claims are handled as workers' compensation, and the Department of Labor handles those. She was also informed that several laws dictate the return of sick leave hours to a leave bank. The complainant was informed of ways to contact the Department of Labor to discuss remedying the situation.

Q47. Do you believe age was a factor in this action?

A47. No, the return of sick leave to employees leave bank depends on several federal laws and executive actions. As far as I am aware Indian health services does not have any say in the return oven employees leave under workers' compensation.

Q48. Do you believe color was a factor in this action?

A48. No, the return of sick leave to employees leave bank depends on several federal laws and executive actions. As far as I am aware Indian health services does not have any say in the return oven employees leave under workers' compensation.

Q49. Do you believe race and national origin was a factor in this action?

A49. No, the return of sick leave to employees leave bank depends on several federal laws and executive actions. As far as I am aware Indian health services does not have any say in the return oven employees leave under workers' compensation.

Q50. Do you believe disability (mental/physical) was a factor in this action?

A50. No, the return of sick leave to employees leave bank depends on several federal laws and executive actions. As far as I am aware Indian health services does not have any say in the return oven employees leave under workers' compensation.

Q51. Do you believe reprisal (opposing discrimination) was a factor in this action?

A51. No, the return of sick leave to employees leave bank depends on several federal laws and executive actions. As far as I am aware Indian health services does not have any say in the return oven employees leave under workers' compensation.

Q52. Did the Complainant report to or ask her supervisor(s) or persons responsible why her sick leave was never put back into the E-Comp leave system? If so, who did she ask and what response did she receive?

A52. The complainant was informed on several occasions that whether sick time would be placed back into her leave account is determinant on which federal law was in effect at the time of her illness, and determination was not made by the agency but rather by the Department of Labor as they are responsible for all Worker's Compensation claims of federal employees. This was discussed on several occasions with the complainant and myself. Information on worker's compensation benefits for Federal employees related to COVID-19 was shared with her, and she was informed that more information could be found on the Department of Labor's Division of Federal Employees' Compensation page.

Q53. Why was the Complainant's sick leave never put back into the E-Comp leave system? What is the agency protocol for this situation?

A53. I do not know I defer to the Department of Labors Office of Workers Compensation as they handle these claims. I do not have this information as to why her sick leave was not reinstated. Again, The U.S. Department of Labor's Office of Workers' Compensation Programs (OWCP) administers four major disability compensation programs which provide to federal workers (or their dependents) and other specific groups who are injured at work or acquire an occupational disease.

Q54. What is the agency protocol for this situation?

A54. Employees file claims through the ECOMP system.

Claim 6: Complainant alleged she was subjected to discrimination based on race (American Indian), color (dark), age (Y.O.B. 1965), national origin (member of Omaha Tribe of Nebraska), disability (physical/mental), reprisal (opposing discrimination) and subjected to harassment (non- sexual)/hostile work environment when:

Complainant alleged she was subjected to harassment/hostile work environment. Examples include, but not limited to: Complainant alleged she was told she needed to provide a doctor's note for her immunocompromised status and reasons that she should not be placed on front line RN triage. Complainant further alleged other Caucasian and contract employees were not required to submit this documentation.

Q55. For this alleged discrimination, please identify: what happened, when it happened, and who made it happened, and the nature of your involvement. Please be very detailed.

A55. All reasonable accommodation requests go through the equal employment opportunity office at Phoenix area office. Complainant was instructed to contact that office as they stated they did not have a reasonable accommodation on file. The process is the same for anyone who request a reasonable accommodation. The complainant did fail to participate in the interactive process. The complainant was requested to provide form HHS – 813 and Reasonable Accommodation Medical Request Form as provided by the ADR Reasonable Accommodation Program Manager at Phoenix area EEO office. The EEOC states that when the disability and/or the need for accommodation is not obvious, the employer may ask the individual for reasonable documentation about his/her disability and

functional limitations. The employer is entitled to know that the individual has a covered disability for which s/he needs a reasonable accommodation. Reasonable documentation means that the employer may require only the documentation that is needed to establish that a person has an ADA disability, and that the disability necessitates a reasonable accommodation. Thus, an employer, in response to a request for reasonable accommodation, cannot ask for documentation that is unrelated to determining the existence of a disability and the necessity for an accommodation. Complainant was informed of the reasonable accommodation process and that the contact for the process was at the Phoenix area office, the was instructed to contact the individual George Patton. The complaint was informed that according to Mr. Patton, there was no approved reasonable accommodation on file. I understand the employee refused to participate in the reasonable accommodation interactive process. I understand that the complainant was asked to provide appropriate documentation from a healthcare provider regarding the nature of any impairment(s), severity, duration, activities limited by the impairment(s), and the extent to which the impairment(s) limits the employee's ability to perform the job's essential duties/functions.

Q56. Do you believe age was a factor in this action?

A56. No

Q57. Do you believe color was a factor in this action?

A57. No

Q58. Do you believe race and national origin was a factor in this action?

A58. No

Q59. Do you believe disability (mental/physical) was a factor in this action?

A59. No

Q60. Do you believe reprisal (opposing discrimination) was a factor in this action?
A60. No

Q61. Were you aware of the nature of the Complainant's medical condition?

A61. The complaint stated that she was immunocompromised because of asthma. Evidence-based materials state that people with asthma are not considered immunocompromised; however, infections may sometimes trigger asthma. People with asthma should know the need to avoid infections and what to do if they become unwell. The agency is allowed to ask for documentation of the employee's disability to understand how to accommodate her best. As far as I am aware that was what was requested and that the employee refused to provide. I can only attest to what I am aware of as the EEO officer handles the Reasonable Accommodation process.

Q62. Did the Complainant file medical documentation from a medical professional regarding their medical condition?

A62. Not that I am aware of. In the past, I believe she filed but failed to participate in the interactive process only to demand that she had one and we could not do certain things as she decided it might cause an issue. I am not aware of an agreed Reasonable Accommodation only her statements many of which she states she cannot do are a requirement for her position.

Q63. Did the Complainant's medical condition (when active) interfere with her ability to perform any of the essential functions of her position? If so, please detail what specific functions of the Complainant's position she was unable to perform, or limited in performing, due to her medical condition.

A63. I do not have sufficient knowledge or information to respond to this question. Not that I can attest to as I have not seen any medical documentation regarding her limitations.

Q64. Did the Complainant request accommodation for her disability? (If yes, please identify the accommodation requested, the reason for request, the date of request, and the Agency's response to the request.)

A64. She states she did, but I do not have sufficient information to respond to this question effectively. I would have to defer to the ADR Reasonable Accommodation Program Manager at the Phoenix area EEO office Mr. George Patton. Any time requested to complete an assignment she did not want to do she

stated she could not because of reasonable accommodation. However, I was informed by the EEO officer that there was not a reasonable accommodation on file.

Q65. During the time of the alleged harassment(s) at issue, were you aware of the Complainant's disability? Explain your answer.

A65. I am still not aware of a disability only an illness of asthma. A diagnosis of asthma by itself does not constitute a disability.

Claim 7: Complainant alleged she was subjected to discrimination based on race (American Indian), color (dark), age (Y.O.B. 1965), national origin (member of Omaha Tribe of Nebraska), disability (physical/mental), reprisal (opposing discrimination) and subjected to harassment (non- sexual)/hostile work environment when: Complainant alleged her reasonable accommodation, which she says was on file since February 21, 2022, was removed. Complainant further alleged she was being required to repeatedly submit forms to put an accommodation back in place; and,

Q66. For this alleged discrimination, please identify: what happened, when it happened, and who made it happened, and the nature of your involvement. Please be very detailed.

A66. As far as I am aware from information provided by the EEO officer Mr. Patton, the employee did not have a reasonable accommodation on file, which was the information provided to the complainant. Along with that information the complainant was provided with the information on whom to contact for a reasonable accommodation.

Q67. Do you believe age was a factor in this action?

A67. No

Q68. Do you believe color was a factor in this action?

A68. No

Q69. Do you believe race and national origin was a factor in this action?

A69. No

Q70. Do you believe disability (mental/physical) was a factor in this action?

A70. No

Q71. Do you believe reprisal (opposing discrimination) was a factor in this action?

A71. No

Q72. Was the Complainant's reasonable accommodation that was on file since February 21, 2022, removed from the request system or from any files?

A72. As previously stated, I am unaware of an approved reasonable accommodation or any files being removed from the system. Additionally, managers would not have access to the EEO system.

Q73. Who removed the reasonable accommodation that was on file since February 21, 2022? What was the reasoning given for the removal of the reasonable accommodation? Was any alternative reasonable accommodation offered by the agency?

A73. I have no information regarding this question. I would not have access to a system to remove any files or documentation. Nor am aware of the communications between the complainant and the EEO officer in regards to a reasonable accommodation. I have no knowledge of an agreed Reasonable Accommodation. The participant refused to participate in any form of interactive process when she stated had a reasonable accommodation, all she would do is state is she had one and we could not make her see patients which is a requirement of the position she occupies. She would not discuss what she felt to be reasonable and only stated that we had to make accommodations.

Q74. What forms was the Complainant asked to repeatedly submit? Who asked her to repeatedly submit forms to put a reasonable accommodation back in place after the removal of her reasonable accommodation request? Why as she asked to repeatedly submit forms? When was each new form submitted (to include dates)?

A74. The complainant was asked to complete the Department of Health and Human Services, Indian Health Service, Letter Requesting Documentation for Determining ADA Eligibility from a Medical Provider as provided by the EEO officer. As far as I know, this is the only form she was ever asked to complete. The complainant was informed that by refusing to provide documentation or participate in the interactive process, the agency risks providing an accommodation that is ineffective for her or improperly denying reasonable accommodation because it is unaware of alternatives that may need. Under the ADA, when an employee requests an accommodation and the disability and need

for accommodation are not obvious, the employer can request medical documentation to help determine whether the employee has a disability and needs the requested accommodation and information to help process the accommodation request. I do not consider any request to be repeatedly if the complainant failed to complete the paperwork necessary when it was originally requested. She was requested to complete the same paperwork everyone else is required to complete if the disability is not obvious in accordance with Federal Law.

Claim 8: Complainant alleged she was subjected to discrimination based on race (American Indian), color (dark), age (Y.O.B. 1965), national origin (member of Omaha Tribe of Nebraska), disability (physical/mental), reprisal (opposing discrimination) and subjected to harassment (non- sexual)/hostile work environment when: Complainant alleged she was being required to swab symptomatic respiratory patients which was not part of her official duties.

Q75. For this alleged discrimination, please identify: what happened, when it happened, and who made it happened, and the nature of your involvement. Please be very detailed.

A75. Patient care is part of the complainant's official duties as a licensed registered nurse. All registered nurses in the primary care clinic that were not contractors were required to rotate days to do nasal swabs for COVID-19 testing. Contractors were not required as aas I understand that was not part of the contract's statement of work. When the statement of work was clarified even the contract RN was added to the rotation. See the email from Jason White dated 01/19/2023 to the CCCs.

Q76. Do you believe age was a factor in this action?

A76. No

Q77. Do you believe color was a factor in this action?

A77. No

Q78. Do you believe race and national origin was a factor in this action?

A78. No

Q79. Do you believe disability (mental/physical) was a factor in this action?

A79. No

Q80. Do you believe reprisal (opposing discrimination) was a factor in this action?

A80. No

Q81. Who required the Complainant to swab symptomatic respiratory patients?

A81. I assigned her the task.

Q82. Was this a part of the Complainant's job duties? If not, whose job duty is it to swab symptomatic respiratory patients?

A82. The complainant's official position description states "Uses appropriate medical supplies and/or equipment for the assigned clinic, such as 12-lead EKGs, fetal monitor, pulse oximeter, etc. and ***collects patient specimens***, including blood

draw, if needed. Processes lab specimens for transport or perform tests, such as waived tests, to obtain results." Therefore it is part of complainants job duties.

Q83. Please identify any witnesses with relevant information regarding the claims accepted for investigation, including name, contact information and a brief description of what relevant information they can provide.

A83.

- ADA Reasonable Accommodation Program Manager at Phoenix Area EEO Office Mr. George Patton Reasonable accommodation requests. George.Patton@IHS.gov
- Jason White Nurse Consultant Jason.White@ihs.gov
- Brenda Barr Clinical Nurse Brenda.Barr@ihs.gov
- Elizabeth Hill Clinical Care Coordinator Elizabeth.Hill@ihs.gov
- Bertha Edison Clinical Care Coordinator Bertha.Edison@ihs.gov
- Angela Dickerson Supervsory Clinical Nurse Angela.Dickerson@ihs.gov
- Carolyn Taphae Supervisory Clinical Nurse Carolyn.Tapahe@ihs.gov

Q84. Do you have any documentation you would like to provide in support of your testimony? If yes, identify all documents you will be providing and their relevance to the complaint.

A84.

1. *Official Position Description PX3439 PCMC CCC (02.28.21)*

2. Department of Health and Human Services Indian Health Service Letter Requesting Documentation for Determining ADA Eligibility from a Medical Provider Form.
3. Letter to Tonie Greve via secure email requesting medical documentation as EEO Officer George Patton requested
4. Email chain regarding CCC split dated January 4, 2023. Relevant to the number of providers assigned to each CCC.
5. Email Jason White on RN COVID swabbing rotation showing all Registered Nurses were assigned a day to do the swabs relevant as it shows that the duty was assigned to all RNs in the clinic.

Q85. Is there anything else you would like to add into the record?

A85.

1. The complainant persistently exploits the Equal Employment Opportunity (EEO) system to retaliate against any person who assumes the role of her supervisor within the Primary Care Clinic. A repetitive cycle of lodging similar or identical accusations of discrimination against each of her supervisors is evident, despite her awareness that her claims are fictitious and lacking in validity. The complainant has consistently misused the EEO system for reasons other than the intended objective of preventing and eliminating discriminatory practices. The case satisfies the criteria for demonstrating a clear pattern of EEO system misuse as stipulated under 29 C.F.R. § 1614.07:

(i). Evidence of recurrent complaints.

(ii). Claims that are alike, identical, lack detail or pertain to previously resolved issues.

(iii). Proof of bypassing other administrative procedures, retaliating against the organization's internal administrative process and overloading the EEO complaint system of the agency.

The facts reveal that the complainant's sole aim is to lodge unavailing EEO complaints against the organization in vengeance for being held responsible for accomplishing the tasks specified in her official job description. The individual was always treated equivalently to all other employees I supervised, irrespective of their job title, race, age, color, religion, gender, nationality, disability, or tribal association. Until this claim was made, I was unaware of her tribal affiliation or that of Ms. Hill or Ms. Edison.

The complainant currently has five Hostile Work Environment allegations pending against her.

2. The complainant continues exploiting the EEO system as retribution against anyone who oversees her within the clinic. The complainant has consistently lodged similar or duplicate accusations of bias against every manager she's worked under, fully cognizant that these claims are groundless and fabricated. The complainant has demonstrated a blatant pattern of EEO system abuse, using it for objectives outside its intended use of discrimination prevention and elimination. Again the grievance satisfies the criteria for demonstrating a manifest misuse of the EEO process as delineated under 29 C.F.R. § 1614.07:

(i). Documentation of multiple grievances.

(ii). Accusations that are comparable, identical, lack detail, or pertain to previously settled issues.

(iii). Proof of bypassing other administrative procedures, retaliating against the organization's internal administrative process and overstressing the agency's EEO complaint system.

The facts indicate that the complainant's sole aim is to lodge pointless EEO allegations against the supervisor in retribution for her being held responsible for fulfilling her official role obligations. The complainant was never treated differently from any other employees I managed, irrespective of their job title, race, age, color, religion, sex, national origin, disability, or tribal association. Until this allegation came up, I had no knowledge of her tribal affiliation or that of Ms. Hill or Ms. Edison.

The employee currently has five pending Hostile Work Environment charges against her.

3. The complainant's abuse of the EEO process is a clear attempt to harass and intimidate the agency. The complainant's repeated filings of frivolous complaints are a waste of the agency's resources and time. They also serve to undermine the integrity of the EEO process.

For these reasons, I respectfully request a dismissal of the complainant's latest complaint for:

- **Duplication of a claim**: The allegation states a claim identical to one that was raised in a pending complaint or in a prior complaint that has been decided. 29 C.F.R. Section 1614.107(a)(1).
- **Mootness**. There is no reasonable expectation that the action or practice will happen again and interim relief or events have completely and irrevocably eradicated the effects of the alleged violation. 29 C.F.R. Section 1614.107(a)(5).
- **Abuse of process:** The complaint evidences a clear pattern of misuse of the EEO process for a purpose other than the prevention and elimination of employment discrimination. A clear pattern of misuse of the EEO process requires: (1) evidence of multiple complaint filings; and (2) allegations that are similar or identical, lack specificity, or involve matters previously resolved; or evidence of circumventing other administrative processes or intentionally trying to clog the agency's in-house administrative processes or overburdening the EEO complaint system. 29 C.F.R. Section 1614.107(a)(9).

Additionally, I ask that the complainant be warned about any further acts of contumacious conduct. Reference case Agency. Stoyanov v. Dep't of the Navy, EEOC Appeal Nos. 0120113142, 0120113817, & 0120114019 (December 6, 2011).

COUNTER-COMPLAINTS

Defamation (Slander and Libel), abuse of process, harassment, hostile work environment, contumacious conduct, intentional infliction of emotional distress, and racial and gender discrimination.

Complainants' statements were made in writing and orally.

The Complainant's statements were actual malice as the Complainant made false statements about me, knowing they were false and/or with reckless disregard for whether the statement was true or false.

The complainant was negligent about whether her statements were true or false.

As a result of the statements made against me, I have suffered the following:

- Loss of reputation: I have been publicly accused of wrongdoing, which has damaged my reputation.
- Damage to my career: I have been relieved of my supervisory duties in three departments as a result of the accusations against me.
- Mental anguish: I have experienced severe emotional distress as a result of the accusations against me. I have had trouble sleeping, concentrating, and enjoying my life.
- Emotional distress: I have been humiliated and shamed by the accusations against me. I have felt isolated and alone.
- Public humiliation: I have been the subject of gossip and speculation at work and in my community. I have felt embarrassed and ashamed.
- Loss of enjoyment of life: I have lost my zest for life. I no longer enjoy the things I used to do.
- Depression: I have depression. I have had difficulty sleeping, eating, and concentrating.
- Anxiety: I have anxiety. I am constantly worrying about the future and what people think of me.
- Loss of appetite and energy: I have lost my appetite and have difficulty getting through the day. I feel tired and run down all the time.
- Sleep disturbance: I have difficulty sleeping. I wake up in the middle of the night and have trouble falling back asleep.

I am requesting that action be taken to protect my rights and to ensure that I am not discriminated against and harassed in the future by the complainant.

I typed the forgoing Affidavit consisting of 33 pages and hereby attest under penalty of perjury that it is true and correct to the best of my knowledge and belief. I have signed or initialed each page. I have been allowed to make any corrections and/or additions I have submitted.

Wednesday, June 28, 2023

Kevin D. Turnage
Affiant Name

Date

Name
EEO Investigator

Date

**********NOTHING BELOW THIS LINE**********

From: Turnage, Kevin D. (IHS/PHX)
Sent: Wednesday, January 4, 2023 09:48
To: Greve, Tonie (IHS/PHX) <Tonie.Greve@ihs.gov>
Subject: RE: CCC Provider list

It is divided in a way to make it equal by number patients not number of providers or specialty. Remember that you will not be following all of these providers patients only patients on your panel much different than what is currently done. I know Pogo is Temporary but she has taken on Dr. Skantharoopan Panel until a replacement is hired.

V/r

Hon. Kevin D. Turnage MSN Ed. RN, AMB-BC, NHDP- BC, NPD-BC
SCN for PCMC Employee Health and Behavioral Health

From: Greve, Tonie (IHS/PHX) <Tonie.Greve@ihs.gov>
Sent: Wednesday, January 4, 2023 9:44 AM
To: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>
Subject: RE: CCC Provider list

Good Morning Kevin;

It looks like I would have Hudson Internal- Med, Miles Internal- Med.
Pogo is temporary and Internal-Med.
Internal Medicine doctors put out the most referrals. Can we all have an Internal-Medicine Doctor on each team?

Just a suggestion.
Respectfully,
Tonie

From: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>
Sent: Wednesday, January 4, 2023 9:32 AM
To: Greve, Tonie (IHS/PHX) <Tonie.Greve@ihs.gov>; Hill, Elizabeth I (IHS/PHX) <Elizabeth.Hill@ihs.gov>; Edison, Bertha (IHS/PHX) <Bertha.Edison@ihs.gov>
Cc: White, Jason (IHS/PHX/PIMC) <Jason.White@ihs.gov>
Subject: CCC Provider list

Below is the new proposed split of providers to CCC. I have tried to average out by number of patients instead of by team, of course this will change as new CCC's come on board. Please review and reply with any concerns/comments at your earliest.

Greve		Edison		Hill	
Black	980	Goswami	676	Baek	129
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Bellantoni	274	Ngo	986	White	1039
Miles	846	Pogo	779	Phan	964
Tsiningine	1037	Richardson	923	Pinsonneault	1061
Hudson	943	Larosa	783	Sahai	932

Total	**4942**	**TOTAL**	**5017**	**TOTAL**	**5050**

Hon. Kevin D. Turnage RN, MSN Ed. AMB-BC, NHDP- BC, NPD-BC
Supervisory Clinical Nurse PCMC and Employee Health , COR II,
Phoenix Indian Medical Center
602-263-1574
Cell: 602-647-9126

"I am not authorized to obligate the government for any work, service or product. Please do not perform any work that will require payment without a valid contract, issued by a government Contracting Officer. Changes to a contract can only be made by the contracting officer. Contractors are reminded to promptly notify the contracting officer of any conduct that may be regarded as a change to the contract in accordance with FAR52-243-7"

Subject: RE: CCC Provider list
Date: Wednesday, January 4, 2023 at 9:56:40 AM Mountain Standard Time
From: Greve, Tonie (IHS/PHX)
To: Turnage, Kevin D. (IHS/PHX)
Attachments: image001.png, image002.jpg, image003.jpg, image004.jpg, image005.jpg, image006.jpg, image007.png

Good Morning Kevin;

Ok, I know that there will be changes down the pike later, but there will be changes for PCP based on the doctors specialty, based on patients request.
Will these doctors change when the new CCC is hired?

Please keep me updated with all the new process changes so that I can perform the duties.

Respectfully,
Tonie Greve RN CCC Silver & Ocotillo Teams

From: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>
Sent: Wednesday, January 4, 2023 9:48 AM
To: Greve, Tonie (IHS/PHX) <Tonie.Greve@ihs.gov>
Subject: RE: CCC Provider list

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SCN for PCMC Employee Health and Behavioral Health

From: Greve, Tonie (IHS/PHX) <Tonie.Greve@ihs.gov>
Sent: Wednesday, January 4, 2023 9:44 AM
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Just a suggestion.
Respectfully,
Tonie

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Sent: Wednesday, January 4, 2023 9:32 AM
To: Greve, Tonie (IHS/PHX) <Tonie.Greve@ihs.gov>; Hill, Elizabeth I (IHS/PHX) <Elizabeth.Hill@ihs.gov>; Edison, Bertha (IHS/PHX) <Bertha.Edison@ihs.gov>
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Total	**4942**	**TOTAL**	**5017**	**TOTAL**	**5050**

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Subject: RE: CCC Provider list
Date: Wednesday, January 4, 2023 at 9:44:08 AM Mountain Standard Time
From: Greve, Tonie (IHS/PHX)
To: Turnage, Kevin D. (IHS/PHX)
Attachments: image007.png, image008.jpg, image009.jpg, image010.jpg, image011.jpg, image012.jpg, image013.png

Good Morning Kevin;

It looks like I would have Hudson Internal- Med, Miles Internal- Med.
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Internal Medicine doctors put out the most referrals. Can we all have an Internal-Medicine Doctor on each team?

Just a suggestion.
Respectfully,
Tonie

From: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>
Sent: Wednesday, January 4, 2023 9:32 AM
To: Greve, Tonie (IHS/PHX) <Tonie.Greve@ihs.gov>; Hill, Elizabeth I (IHS/PHX) <Elizabeth.Hill@ihs.gov>; Edison, Bertha (IHS/PHX) <Bertha.Edison@ihs.gov>
Cc: White, Jason (IHS/PHX/PIMC) <Jason.White@ihs.gov>
Subject: CCC Provider list

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Hon. Kevin D. Turnage RN, MSN Ed. AMB-BC, NHDP- BC, NPD-BC
Supervisory Clinical Nurse PCMC and Employee Health , COR II,
Phoenix Indian Medical Center
602-263-1574

Cell: 602-647-9126

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Subject: Panel Changes
Date: Wednesday, January 4, 2023 at 9:07:56 AM Mountain Standard Time
From: Greve, Tonie (IHS/PHX)
To: Turnage, Kevin D. (IHS/PHX)

Good Morning Kevin;

I am checking on the status of the panel changes or if they are going to stay the same of me having the Silver & the Ocotillo Teams.
Reason: I am getting forward notifications after providers/doctors send to other team members requesting assistant from the other CCC who then forwards to me to complete the request.

Do you know when or if there will be changes to the assignments of panels, or doctors?

Respectfully,
Tonie Greve RN CCC Silver & Ocotillo Teams.

Subject: Respiratory Swab Rotation

Date: Thursday, January 19, 2023 at 9:17:45 AM Mountain Standard Time

From: White, Jason (IHS/PHX/PIMC)

To: Greve, Tonie (IHS/PHX), Hill, Elizabeth I (IHS/PHX), Edison, Bertha (IHS/PHX), Barr, Brenda (IHS/PHX), Frost, Arlene (IHS/PHX/PIMC) [C]

CC: Turnage, Kevin D. (IHS/PHX)

Starting on Today 01/19/2023 PCMC nurses will the respiratory swab rotation as follows:

Tonie Greve: Monday
Brenda Barr: Tuesday
Elizabeth Hill: Wednesday
Bertha Edison: Thursday
Arlene Frost: Friday

Lab draws will still be completed in our Lab Room until the Lab gets their isolation room set-up.

The process will be as described by Dr. Baek. There is the possibility that a patient does not get scheduled in BPRM. If you receive a call on the day that you are scheduled, that a COVID swab needs to be done, it is an expectation that you swab the patient.

Jason White BSN, RN, CEN, CPEN, CCRN, CFRN

Nurse Consultant & RN Retention and Recruitment Analyst

Nursing Administration

Phoenix Indian Medical Center

602-581-6331

Do you care to be the layer of the bricks that seal your fate? Or would you rather be the architect of what we might create?

CONFIDENTIALITY NOTICE: This email communication may contain private, confidential, or legally privileged information intended for the sole use of the designated and/or duly authorized recipient(s). If you are not the intended recipient or have received this email in error, please notify the sender immediately by email and permanently delete all copies of this email including all attachments without reading them. If you are the intended recipient, secure the contents in a manner that conforms to all applicable state and/or federal requirements related to privacy and confidentiality of such information

Turnage, Kevin D. (IHS/PHX)

From: Turnage, Kevin D. (IHS/PHX)
Sent: Friday, October 7, 2022 1:43 PM
To: Greve, Tonie (IHS/PHX)
Cc: White, Jason (IHS/PHX/PIMC); Lee, Candace S. (IHS/PHX); Homer, Michelle (IHS/PHX); Patton, George W (IHS/PHX)
Subject: RE: reasonable accommodation denied?
Attachments: RA Medical Request Form_AttAB46.tmp.H.S_.pdf; HHS-813 Confirmation of Request for Reasonable Accomodation.pdf

As previously stated the only reasonable accommodation for I am aware of is for Asthma/immunocompromised requesting to limit your time in the triage room. When we discussed previously you stated that there were things in the triage room that aggravated your asthma and I agreed to have the items removed from the room (which has occurred) and also stated that when assigned to cover the triage room you could use a exam room instead. You stated at that time that was agreeable until a finalization of the reasonable accommodation. I understand you state that a licensed clinician signed a letter for you. However, the letter I sent you on June 17, 2022 requested additional information within 15 days of date of receipt of the letter. I am not aware of any requested documentation being returned.

In regards to the October 5th and 6th assignments of staff. It was your turn to cover the triage room so when your co-worker was taken to the Emergency Department I assigned you to cover in the order of rotation. Brenda covered while Arlene was on Vacation and Bertha covered before that making it your turn to cover. Everyone was assigned to work the vaccination and each CCC covers the triage room, therefore, all RN's including the contract RN were assigned the same duties to cover vaccination clinic and the triage room and there were no preferential treatment of any employee.

Since the additional medical documentation was not received as requested, it is my understanding that in order to initiate a reasonable accommodation request, you must complete a new form HHS-813 and return the form to george.patton@ihs.gov or Michelle.Homer@ihs.gov. Please let me know if you have any questions or need any additional information. I am also attaching the medical request form for your provider to complete to help expedite the process.

In regards to your ecomp filing. As previously stated on May 18, 2022 when I checked last and discussed with you, the supervisor completed the filing. If you have any questions regarding the filing, you should check with the Department of Labor. I am not sure who Leonard is that you are referring to so, please provide me with full name and date you spoke with him.

V/r

Hon. Kevin Turnage MSN Ed. RN, AMB-BC, NHDP-BC, NPD-BC

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From: Greve, Tonie (IHS/PHX) <Tonie.Greve@ihs.gov>
Sent: Thursday, October 6, 2022 4:01 PM
To: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>

Cc: White, Jason (IHS/PHX/PIMC) <Jason.White@ihs.gov>; Lee, Candace S. (IHS/PHX) <Candace.Lee@ihs.gov>
Subject: RE: reasonable accommodation denied?

Good Afternoon Kevin:

I have a signed Reasonable Accommodation letter by a Licensed Clinician, specialists who completed the I.HS/HHS form and turned in to the agency.
I did not refused the mandatory assignment on 10/6/2022 out of fear and retaliation/reprisals.

I was discriminated and it will be addressed through the EEOC process.
As a Federal Employee no one should be discriminated.
All employees who have reasonable accommodations should be treated the same with no preferential treatment.

You were acting in the Primary Care Medicine Clinic Nurse supervisor prior to taking the position permanently and at that time told you of my disability.
There is nothing more that should be supplied for the Reasonable Accommodations form, my immunocompromised status will be for the rest of my life and has been before working at HHS/I.HS.
I left that position Nurse Assessment/Nurse Triage at primary care clinic due to my permanent disability and other discriminatory issues. (I am sharing this information again with management officials)

I am working with you as my supervisor but I do not want to be discriminated, harassed or retaliated.

Thank you for looking into the E-Comp that I completed back in January 2022 of this year which I submitted the required documents.
I was mandated to work in the nurse assessment/nurse triage January 3, 4, 5 and became infected with coronavirus on January 6, 2022 after being exposed to several persons let into the room. I used 57 hours of leave and like I discussed as I know the only person in primary care medicine clinic who has not had the leave restored. I submitted positive covid-19 test, documentation of clinic visits, f/u visit while out sick with the virus. I hope this can be completed it needs a supervisor to go into the system and answer their portion as I was told by Leonard.

Respectfully,

Tonie Greve RN CCC Silver & Ocotillo Teams

From: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>
Sent: Wednesday, October 5, 2022 3:54 PM
To: Greve, Tonie (IHS/PHX) <Tonie.Greve@ihs.gov>
Subject: RE: reasonable accommodation denied?

Tonie,

As far as I am aware, your RA was not denied I am not aware of a clinician reviewing your RA or any documents other than what EEO officer sent to me. If you have additional information or documentation please share. We discussed when I became clinic Supervisor that you were requesting a RA for the triage room due to your Asthma and I agreed that if you had to do triage then you could use an exam room instead of the triage room. I have not received any other requests other than that one for the triage room. If there are other accommodations, you feel you need please let me know so that they can be properly addressed. As explained this morning regarding your ecomp claim I would try and follow-up and asked that you give me until next week to see if I could get a response because since it was filed it is no longer with IHS it should be with the Department of Labor. I do not have the specifics of when you contracted COVID

including dates or specific laws that may or may not have been in effect at that time. I have left voicemail for my contact and will continue to follow-up to try and get you an answer regarding your ecomp claim.

V/r

Hon. Kevin Turnage MSN Ed. RN, AMB-BC, NHDP-BC, NPD-BC

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From: Greve, Tonie (IHS/PHX) <Tonie.Greve@ihs.gov>
Sent: Wednesday, October 5, 2022 8:23 AM
To: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>
Subject: RE: reasonable accommodation denied?

It has been denied per the written statement by the Licensed Clinician.

I was placed in this nurse triage in January 3, 4, 5 and was exposed to positive covid-19 patients and became infected. I filed the E-Comp and used 57 hours of sick leave. All other Federal Employees were given back the hours used for the sick leave except for Tonie Greve. I have asked Angie Dickerson to complete as the supervisor, and asked you to assist in completing.
I still have not had the E-Comp hours returned to my bank. I do not understand why I am the only primary care medicine employee who has not had this completed by a supervisor.

I am not the only persons at primary care medicine clinic that have requested Reasonable Accommodations but the forms I submitted have been denied. I am following your directive but had to ask and clarify.

I don't have anyone covering my official duties so I will have no choice but to stay here and perform double duties.

Respectfully,

Tonie Greve RN CCC Silver & Ocotillo Teams

From: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>
Sent: Wednesday, October 5, 2022 7:50 AM
To: Greve, Tonie (IHS/PHX) <Tonie.Greve@ihs.gov>
Subject: RE: reasonable accommodation denied?

No as we discussed you can use and exam room instead of the triage room. Would you rather go to vaccine clinic and do vaccinations on employees today and tomorrow.

V/r

Hon. Kevin Turnage MSN Ed. RN, AMB-BC, NHDP-BC, NPD-BC

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From: Greve, Tonie (IHS/PHX) <Tonie.Greve@ihs.gov>
Sent: Wednesday, October 5, 2022 7:33 AM
To: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>
Subject: reasonable accommodation denied?

Good Morning Kevin;

Heading over had to grab equipment and things.
I heard your text while driving, could not respond.

I do have a question on the reasonable accommodations that I submitted.
Was it denied?

Respectfully,

Tonie Greve RN CCC Silver & Ocotillo Teams

Turnage, Kevin D. (IHS/PHX)

From: Greve, Tonie (IHS/PHX)
Sent: Thursday, June 16, 2022 8:20 AM
To: Turnage, Kevin D. (IHS/PHX)
Cc: Lee, Candace S. (IHS/PHX)
Subject: RE: Notification

Kevin;

I do have reasonable accommodations for my Lung Disease.
Since onboarding and when asked I have forms submitted from the Agency by listing my disabilities. Working here for 11 years, you may have received the information in a email. I do respond and list my disabilities.

2. This information was given to Rebecca and I have asked repeatedly to not have 3 or more broadcast sent at the same time.
As the Direct supervisor you should have been given the copy of the reasonable accommodations for my lung disease and the coverage for the Nurse Assessment/Triage Room. I did speak to you about the E-Comp which I still have not heard back on status from you which as immunocompromised I was placed in the nurse assessment room repeatedly, not a shared duty with all team CCC, contracted Covid.
Broadcast messages sent in sets of 3 or more is a disruption, especially if I am typing a telephone conversation with a patient that I have on the phone and it stops it immediately, there is evidenced base literature and data that can support this as a interruption.

3. I do complete the task but <u>where does it say any email today or in the past that I am the only person to schedule for the Whole Primary Care Medicine Clinic?</u>
Rebecca included all in the email, she initiated it for all team leader to be aware.

4. I have seen where patient information has been broadcasted to all Phoenix Indian Medical Center, and I have had training from this service unit not to do broadcast due to the reasons that if a person logs off it will go out to everyone who is logged on. That is a Hippa violation and a not a Need to Know.
The skype messages, have been addressed when I get it that has patient identification in it, I do inform those who send it not to use it to please put the information in a notification and I will review the information, so to protect the patients identity, this as well if I receive outside emails from patients.
(EBM) has that been approved by Indian Health Service and brought to the attention to HIM Director?

6. I do work effectively and go above an beyond to assist the patients that Indian Health Service Serves. I am held accountable not only for my Official Duties but that of the other team members who shop while at work, watch movies on their phones, turn off their phones, not answer their phones, you have repeatedly sent out emails to staff prohibiting these types of practices, but it still continues.
I do carry out the task of scheduling the appointments not only for the Phoenix Indian Medical Center Case Managers but for patients who repeatedly can not reach anyone else, I have numerous Doctors from within Phoenix Indian Medical Center who contact me as well as other departments as they can not reach the persons who holds official positions but do not respond to the calls or broadcast or skype messages.
Page 2 Paragraph 2 does list my duties but once again it does not say that I am to cover the Whole Primary Care Medicine Clinic, and I did write that all have the same job description listed on the Official Position Description, that is true. I carry out my official duties daily unless on sick leave or vacation, then return to complete the duties that were not picked up by others even when patients repeatedly try to reach the employees who are on their phones. This was in your previous emails.

You Hold me accountable to do my official duties, and (other duties as assigned), your disparate treatment is very evident from this email, you have also have shown and made racist comments to me and disparate treatment is very evident.

I do try to resolve problems at lowest and communicate, only to get this email response or email response in the past. Now you want me to complete a reasonable accommodations for this simple request which I will contact George Patton to complete

I am including the next line in my chain of command as your hostility towards me as well as the racist comments leads me to this so that all are aware.
I am more than willing to sit down with anyone to find resolution that is my professionalism.

Respectfully,

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To: Greve, Tonie (IHS/PHX) <Tonie.Greve@ihs.gov>
Cc: Lee, Candace S. (IHS/PHX) <Candace.Lee@ihs.gov>; Patton, George W (IHS/PHX) <George.Patton@ihs.gov>
Subject: Re: Notification
Importance: High

Tonie,

I have removed Mrs. Carrillo and Ms. Tapahe from this email chain since they do not need to be part of this conversation. I have also added George Patton as I am asking him to provide me with the information on any Reasonable Accommodations that the agency has agreed to since I am not aware of any accommodations, and none have been shared with me, so I was unaware of any conditions up until I received this email. There are several issues you have raised in this email, and I am going to attempt to address them systematically.

1. I understand that the changes being made can be frustrating and difficult to adjust to. Please know that the changes are being made for the good of our patients and have been researched and are evidence-based.
2. You state in your email " I do have ADHD; the agency and you are aware as I disclosed this to you." Me and you have never had any type of conversation regarding any of your medical problems or accommodations so I am not sure who this statement is directed to, but I am your supervisor and should be the one you are having these conversations with. With that caveat being stated the Job Accommodation Network lists reasonable accommodations for ADHD to include providing of private workspace (which the agency has provided), additionally recommends providing uninterrupted work time (which can be accomplished but shutting your office door) the broadcast messages are not considered an interruption as they are part of your duties and requirements for accomplishing your duties. Is there anything else we can provide to assist with accommodating you? Recommendations include to-do lists, assistance with prioritization, the use of EAP and structured breaks. I am available to discuss options if you wish.

3. Regarding notifications from inpatient case management, we had a conversation via email about this on May 9, 2022, when I sent out directives on how the inpatient case managers were to notify PCMC employees on the need for appointments for both emergency room and hospital discharges. At which time I informed you regarding this process "This is a new process and may have to be changed but we can discuss as a group to determine if there is a better more efficient way to ensure that CCC's are helping to get follow-up discharge visits scheduled prior to discharge. The broadcast serves as notice of the need of your assistance with scheduling." You stated there was not a reason to have a conversation regarding the process, specifically you stated, "Why would we discuss this in a group when a process has been used in the past?" I informed you that this was a management directive that was expected to be followed. You have not made any mention of this directive again until today when you sent the initial email to Ms. Carrillo and she informed you that she was following the directions given to her by me on how to contact CCC's and the Empanelment nurse (which are still the same as when we discussed this on May 9, 2022 at the time you said that this directive "this seems more of harassment, bullying not assisting patients, and if prevents me from doing my job. I don't get this broadcast action from any other PIMC or PCMC team member." I explained that the current processes were not working, causing patient delays, and must be changed. The process is the same for the other RN's in PCMC and is not any different from what you are being asked to do and is the equitable expectation of your position and all Clinical Care Coordinator positions.
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5. There would not be evidence-based material (EBM) on the use of broadcasting in our EHR since it is proprietary to the government however there is a plethora of EBM on the use of broadcasting to improve communication between teams, patients, and families. I am willing to assist you in finding some of the most current EBM regarding broadcast messaging in the ambulatory setting.
6. You responded to information I sent out on May 6, 2022, regarding BPRM and the scheduling of patients and claimed that you were being required to take on additional duties not in your position description and blaming other individuals that do not have any say or control in how I am running primary care. I responded to your questions as requested and explained my philosophy and my expectations for the CCC's and how AHRQ considers care coordination to be a necessity to properly care for our patients. I explained the philosophy I am trying to instill at PCMC as "Our philosophy is to help our patients achieve the highest level of wellness and independence. By working together with other team members, we can meet our patients' and family's' healthcare needs. If we are saying that is not my job, then we are not focused on the patient." As stated in my email, this may

be temporary, and some duties as assigned are due to staffing issues. At times your email responses have the appearance of being hostile and threatening in tone and demeanor as previously stated when discussing your official position description and I informed you that the position description for Clinical Care Coordinator (PCMC) on page 2 paragraph 2 beginning on line 7 states that the CCC "Assists with obtaining lab results, medical tests, discharge summaries and other information within the hospital and other facilities and assists with arranging appointments, procedures and discharge planning." To wit you did not respond or request clarification. This email was the first mention of the issue again.

7. The response to Ms. Carrillo has the appearance of being unprofessional, rude, and hostile. I have sent these types of emails in the past and have had to learn from them and how to make changes so as not to give that appearance and I can provide you some of the advice and tips given to me to assist you in avoiding making some of the same mistake I made. On May 5, 2022, I asked you that when there were issues "In the future, please send these types of issues to me or just me and leadership" and not to involve others unnecessarily as it creates a hostile work environment and creates more issues than just the subject you are addressing. I again ask not to include entire buildings or departments when there is an issue especially since there are numerous changes occurring rapidly and you may not be aware of the bigger picture. As stated, several times since I took over PCMC on March 28, 2022, I value your input and insight to the way things are occurring, I am asking that in the future you give me an opportunity to explain my rationale and rectify any issues or perceived issues without involving others.

I hope I have answered all your questions and concerns. As always do not hesitate to contact me if there are any further questions or concerns in which I can assist.

Very Resptfully,

Hon. Kevin D. Turnage MSN Ed. RN, AMB-BC, NHDP-BC, NPD-BC

From: Greve, Tonie (IHS/PHX) <Tonie.Greve@ihs.gov>
Date: Wednesday, June 15, 2022 at 3:09 PM
To: Carrillo, Rebecca J. (IHS/PHX) <Rebecca.Carrillo@ihs.gov>, Tapahe, Carolyn R. (IHS/PHX) <Carolyn.Tapahe@ihs.gov>
Cc: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>, Lee, Candace S. (IHS/PHX) <Candace.Lee@ihs.gov>
Subject: RE: Notification

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You put Un-assigned patient in a notification and I completed the task. I zone in on new notifications from others who do not work at primary care medicine clinic.

HIPAA compliance, if you or anyone sends a notification and I log off on the Electronic Health record the patient's identification goes to anyone who is logged onto the Electronic Health Record, and it shows the patient's identification chart number and the illness.
All can disagree with my pointing this out but not everyone logged into Electronic health record is a **"Need to Know".** This is under HIPAA compliance under the Indian Health Manuel and under HIPAA rules & regulations.

Sending broadcast after broadcast is not faster getting any appointments but it blocks me from performing my job**, there is no evidenced based literature or data** to support this type of scheduling appointments.

I do have ADHD, the agency and you are aware as I disclosed this to you. Professionally I asked all of the inpatient case managers to send a notification or call me, I do answer my phone and if I am on with a patient , I return calls.
Your discriminating against me, it's a form of harassment (non-sexual) since I asked you not to do this, but you continue to do so.

I also have (2) panels 8 to 10 doctors, providers that I am working with daily on phone or even person I huddle with team members.

Thank you for responding;

Respectively,

Tonie Greve RN CCC Silver & Ocotillo Team

From: Carrillo, Rebecca J. (IHS/PHX) <Rebecca.Carrillo@ihs.gov>
Sent: Wednesday, June 15, 2022 2:38 PM
To: Greve, Tonie (IHS/PHX) <Tonie.Greve@ihs.gov>; Tapahe, Carolyn R. (IHS/PHX) <Carolyn.Tapahe@ihs.gov>
Cc: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>; Lee, Candace S. (IHS/PHX) <Candace.Lee@ihs.gov>
Subject: RE: Notification

I'm just following the instructions Kevin Turnage provided us to "Notify CCC via broadcast or phone" according to the standardized algorithm sheet that he sent to our team. If the guidelines have changed please update the standardized algorithm sheet for our group. I apologize, but on our end it can be confusing as changes are made without sufficient communication.
Thank you, Rebecca

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Subject: Notification

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Professionally, I do not work daily by getting broadcast after broadcast, that is not how I was trained nor work daily at Primary Care Medicine Clinic, by doing this repeatedly, you are forcing the way you work onto me without consideration.

Can we find common ground to assist the patients we serve here at Phoenix Indian Medical Center? These actions are more bullying and intimidation.

Can we collaborate and work together? I do have (2) panels and work with (8) doctors daily, as well as get notifications for the Un-assigned team, and the Specialty Clinic.

If the patient is not heading out the door, a notification can be reviewed and an appointment made. I have repeatedly scheduled appointments only to find out the patients were not discharged and the dates of the scheduled appointments do not fall within the 7-10 days.

I review the notifications throughout the day zoning in on the request from persons that do not work at primary care medicine clinic, and it is very distracting when I get broadcast instead of notifications especially when on a phone with a client and typing verbatim what the patient is saying my normal reaction is to delete the broadcast.

I am asking you once again professionally, can you put the request in a small notification, or call me, the broadcast has no information other than needing an appointment.

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Subject: Re: Notification
Importance: High

Tonie,

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Turnage, Kevin D. (IHS/PHX)

From: Greve, Tonie (IHS/PHX)
Sent: Tuesday, July 12, 2022 2:48 PM
To: Turnage, Kevin D. (IHS/PHX)
Cc: Baek, Aimee (IHS/PHX)
Subject: Panel patients

Good Afternoon Team Leaders;

I have a question that would like addressed.

(3) Clinical Care Coordinators are at Primary Care Medicine Clinic

I see that Bertha RN CCC only has (4) providers on the Agave Team.
Bertha is paneling patients to the Ocotillo Team, why isn't she assigning the patients to her team, she only has four providers on the Agave team.

I thought Brenda Clinical Rn duties were paneling patients?

I am the only Registered Nurse, Clinical Care Coordinator Phoenix Indian Medical Center wide that is assigned (8) providers and then two on the Un-assigned team I am getting workload.

I am asking why the work load is not distributed equally, this disparate work load to me is not fair or equitable for only (1) Rn clinical Care Coordinator to manage.

Respectfully,

Tonie Greve Rn CCC Silver & Ocotillo Teams.

Turnage, Kevin D. (IHS/PHX)

From: Greve, Tonie (IHS/PHX)
Sent: Monday, December 19, 2022 9:10 AM
To: Turnage, Kevin D. (IHS/PHX)
Cc: White, Jason (IHS/PHX/PIMC); Lee, Candace S. (IHS/PHX); Patton, George W (IHS/PHX); Homer, Michelle (IHS/PHX)
Subject: RE: Covid Swabbing Clinic

Good Morning Kevin;

As I explained to you that on the computer I was using could not down load the system, so I used the old way RPMS.
I explained that I asked the Preceptor for the Medical Assistants Sharon for assistance for which I could not access. I then walked up front to let the Contact Reps know how to contact me that I was the person today that is going to do the swab testing in the Am and the PM.

I do have the reasonable accommodations and an email that you and I have communicated with, if you fail to honor the reasonable accommodations that is "Managements Rights" and it is discrimination and I have no further option but to add to the current EEOC that I have with the agency, which I have.

Today I added the laughter you did when I told you that the cold air exacerbates my lung disease where as you just laughed and told me that it should not cause that much problems as I walked from one building to the other but in fact I drove my car parked in the Handicap parking space that I use as an American with Disability for my lung disease.

I tried to resolve this reasonable accommodations with you at the lowest level, but you ignore it over and over again, even laugh at me.
I have the reasonable accommodation form DHS/HHS/I.HS in my permanent health records. If it is removed I will then file with the Office of Civil Rights as no one should be able to remove it from my permanent medical health records.

Respectfully,

Tonie Greve RN CCC Silver & Ocotillo Teams

From: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>
Sent: Monday, December 19, 2022 9:02 AM
To: Greve, Tonie (IHS/PHX) <Tonie.Greve@ihs.gov>
Cc: White, Jason (IHS/PHX/PIMC) <Jason.White@ihs.gov>; Lee, Candace S. (IHS/PHX) <Candace.Lee@ihs.gov>; Patton, George W (IHS/PHX) <George.Patton@ihs.gov>; Homer, Michelle (IHS/PHX) <michelle.homer@ihs.gov>
Subject: RE: Covid Swabbing Clinic

Tonie,

You do have access to BPRM as all CCC's have access. There are no appointments scheduled for today as it stands currently. You can return to your normal office and are done with this duty for today.

Everyone should have entry capability to the North Employee entrance, which is the door you are referring.

As stated in my last e-mail the agency has no reasonable accommodations on file for you. I asked what accommodations you are requesting on 15 July 2022 and you did not respond. I have also attached the e-mail regarding this same issue that was sent to you on 17 Oct 2022 and the request for additional information from you I sent on 23 Jun 2022, which a response was never received; this request had a deadline of 15 days from receipt of the letter. I have attempted diligently to work with you to determine your needs and what exactly you are requesting as a reasonable accommodation with no response other than the statement "I have a reasonable accommodation!" I personally have made more than a diligent and prompt good faith effort to assist with Reasonable Accommodations but you have not responded to any of my previous requests. Under the ADA, if an employee does not provide sufficient medical documentation as requested, the employer does not have a duty to continue the accommodation process. I believe you have failed to provide documentation without a valid reason as required.

V/r

Hon. Kevin Turnage MSN Ed. RN, AMB-BC, NHDP-BC, NPD-BC

"I am not authorized to obligate the government for any work, service or product. Please do not perform any work that will require payment without a valid contract, issued by a government Contracting Officer. Changes to a contract can only be made by the contracting officer. Contractors are reminded to promptly notify the contracting officer of any conduct that may be regarded as a change to the contract in accordance with FAR52-243-7"

From: Greve, Tonie (IHS/PHX) <Tonie.Greve@ihs.gov>
Sent: Monday, December 19, 2022 8:05 AM
To: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>
Cc: White, Jason (IHS/PHX/PIMC) <Jason.White@ihs.gov>; Lee, Candace S. (IHS/PHX) <Candace.Lee@ihs.gov>
Subject: Covid Swabbing Clinic

Good Morning Team Leaders:

I was delegated, detailed to this clinic today, and I do not have the access to BPRM as I am at Primary Care Medicine Clinic trying to get set up for any that maybe assigned to this clinic.
I see the spots for testing are along the fence, and hoping I have access to enter in an out of the door, cold weather does mess with my lungs, sharing hence why I asked for the reasonable accommodations but it is being ignored.

I will ask the MA to assist with BPRM

Starting on Monday 12/19/2022 PCMC will begin Respiratory Swabbing with patients in their vehicles. The nurses will cover this and responsibilities will rotate daily as follows:

Tonie Greve
Brenda Barr
Elizabeth Hill
Bertha Edison

Turnage, Kevin D. (IHS/PHX)

From: Greve, Tonie (IHS/PHX)
Sent: Monday, December 19, 2022 9:28 AM
To: Turnage, Kevin D. (IHS/PHX)
Cc: White, Jason (IHS/PHX/PIMC); Lee, Candace S. (IHS/PHX); Patton, George W (IHS/PHX); Homer, Michelle (IHS/PHX)
Subject: RE: Covid Swabbing Clinic

Hon. Kevin Turnage;

You are being dishonest with this email.
Just like you and Brenda Barr RN were talking today while she in your office, neither of you had on your face masks, I waited until she left then entered your office whereas you immediately placed on your face mask.

I am a professional federal employee only want to come into work to perform my official duties without being discriminated against for being immunocompromised with lung disease and being a American Indian/black federal employee.

I have repeatedly tried to resolve these issues with you at the lowest and you laugh at me, make fun of me being immunocompromised with lung disease. I use my handicap parking due to this disability.
This is not hidden as I cannot help that I have a disability that I was born with.

I was placed in a hazardous position last January 3, 4, 5 2022 whereas staff let in infected respiratory patients into the nurse triage/nurse assessment room whereas I became infected with coronavirus and had missed 56 hours of work and even completed the workers comp documentation whereas you were suppose to see why I was not given back my sick hours. Hundreds of federal employees at this facility (PIMC/PCMC) were given the sick hours back, except for me, you were suppose to follow up and you did nothing but ignore this request as a subordinate under your direct supervision.

This all in email communication with you which can not be hidden or denied.

Respectfully,

Tonie Greve RN Silver & Ocotillo Teams

From: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>
Sent: Monday, December 19, 2022 9:18 AM
To: Greve, Tonie (IHS/PHX) <Tonie.Greve@ihs.gov>
Cc: White, Jason (IHS/PHX/PIMC) <Jason.White@ihs.gov>; Lee, Candace S. (IHS/PHX) <Candace.Lee@ihs.gov>; Patton, George W (IHS/PHX) <George.Patton@ihs.gov>; Homer, Michelle (IHS/PHX) <michelle.homer@ihs.gov>
Subject: RE: Covid Swabbing Clinic

Except you never made a statement telling me “that the cold air exacerbates my lung disease where as you just laughed and told me that it should not cause that much problems as I walked from one building to the other but in fact I drove my car parked in the Handicap parking space that I use as an American with Disability for my lung disease.” In addition, I never laughed. As stated numerous time what are you requesting in the form of a reasonable accommodation? What are you saying you cannot do? Are you saying you cannot have face-to-face contact with patients? I have had airflow increased in all clinic rooms that you would currently use the lab room and the triage room. I have ensured all fragrances where removed from the triage room and reiterated to all staff that cologne and perfumes are not allowed. Therefore, I

am at a loss on actually what you are requesting, you can attempt to add this to an EEOC complaint however, you have failed to meet your requirements under the ADA law to participate in the interactive process.

V/r

Hon. Kevin Turnage MSN Ed. RN, AMB-BC, NHDP-BC, NPD-BC

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Cc: White, Jason (IHS/PHX/PIMC) <Jason.White@ihs.gov>; Lee, Candace S. (IHS/PHX) <Candace.Lee@ihs.gov>; Patton, George W (IHS/PHX) <George.Patton@ihs.gov>; Homer, Michelle (IHS/PHX) <michelle.homer@ihs.gov>
Subject: RE: Covid Swabbing Clinic

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I explained that I asked the Preceptor for the Medical Assistants Sharon for assistance for which I could not access. I then walked up front to let the Contact Reps know how to contact me that I was the person today that is going to do the swab testing in the Am and the PM.

I do have the reasonable accommodations and an email that you and I have communicated with, if you fail to honor the reasonable accommodations that is "Managements Rights" and it is discrimination and I have no further option but to add to the current EEOC that I have with the agency, which I have.

Today I added the laughter you did when I told you that the cold air exacerbates my lung disease where as you just laughed and told me that it should not cause that much problems as I walked from one building to the other but in fact I drove my car parked in the Handicap parking space that I use as an American with Disability for my lung disease.

I tried to resolve this reasonable accommodations with you at the lowest level, but you ignore it over and over again, even laugh at me.
I have the reasonable accommodation form DHS/HHS/I.HS in my permanent health records. If it is removed I will then file with the Office of Civil Rights as no one should be able to remove it from my permanent medical health records.

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Tonie Greve RN CCC Silver & Ocotillo Teams

From: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>
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Cc: White, Jason (IHS/PHX/PIMC) <Jason.White@ihs.gov>; Lee, Candace S. (IHS/PHX) <Candace.Lee@ihs.gov>; Patton, George W (IHS/PHX) <George.Patton@ihs.gov>; Homer, Michelle (IHS/PHX) <michelle.homer@ihs.gov>
Subject: RE: Covid Swabbing Clinic

Tonie,

You do have access to BPRM as all CCC's have access. There are no appointments scheduled for today as it stands currently. You can return to your normal office and are done with this duty for today.

Everyone should have entry capability to the North Employee entrance, which is the door you are referring.

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V/r

Hon. Kevin Turnage MSN Ed. RN, AMB-BC, NHDP-BC, NPD-BC

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Sent: Monday, December 19, 2022 8:05 AM
To: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>
Cc: White, Jason (IHS/PHX/PIMC) <Jason.White@ihs.gov>; Lee, Candace S. (IHS/PHX) <Candace.Lee@ihs.gov>
Subject: Covid Swabbing Clinic

Good Morning Team Leaders:

I was delegated, detailed to this clinic today, and I do not have the access to BPRM as I am at Primary Care Medicine Clinic trying to get set up for any that maybe assigned to this clinic.
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I will ask the MA to assist with BPRM

Starting on Monday 12/19/2022 PCMC will begin Respiratory Swabbing with patients in their vehicles. The nurses will cover this and responsibilities will rotate daily as follows:

Tonie Greve
Brenda Barr
Elizabeth Hill
Bertha Edison

Turnage, Kevin D. (IHS/PHX)

From: Greve, Tonie (IHS/PHX)
Sent: Thursday, December 15, 2022 8:18 AM
To: Turnage, Kevin D. (IHS/PHX)
Cc: Patton, George W (IHS/PHX); Lee, Candace S. (IHS/PHX); White, Jason (IHS/PHX/PIMC)
Subject: RE: Respiratory Swab Rotation

Good Morning Kevin;

Thank you for including all Management officials on this racial discrimination claim that I have brought forward.

George could you please send the EEOC forms to me to start the EEOC process.
Americans with Disability Act ADA-employers may not discriminate against qualified employees with disabilities terms of condition related to employment .
Title VII tolerates no racial discrimination subtle or other wise to assign job duties based on the racial preference.

Your email stated Registered Nurse and this contract Caucasian(white) employee is a registered nurse.

Respectfully,

Tonie Greve RN CCC Silver & Ocotillo Teams

From: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>
Sent: Thursday, December 15, 2022 8:09 AM
To: Greve, Tonie (IHS/PHX) <Tonie.Greve@ihs.gov>
Cc: Patton, George W (IHS/PHX) <George.Patton@ihs.gov>; Lee, Candace S. (IHS/PHX) <Candace.Lee@ihs.gov>; White, Jason (IHS/PHX/PIMC) <Jason.White@ihs.gov>
Subject: RE: Respiratory Swab Rotation

Arlene is a RN however, she is not a CCC she is a contract RN. She has been contracted to do the triage room, it is not discrimination it is a management/business decision and ethnicity or race were not factors in the decision. Other considerations go into how we use a contractor, and some changes require modification of the contract through acquisitions. The other nurses Brenda Barr is included in the rotation as is Elizabeth Hill and Bertha Edison. So yes, all Federal Employee RN's in clinic are in the rotation. I am again including George Patton EEO since there is not a reasonable accommodation on file for you that our agency or I are aware of and have copies. The only thing that I am aware of was you stated you could not be in the triage room due to the fragrances in the room. That issue was addressed by ensuring all fragrances were removed and you were also told that when necessary for you to cover that position you could use an exam room in lieu of setting in that room. What duties are you saying you cannot not complete and are requesting a reasonable accommodation to have modified? Please provide the signed written Reasonable Accommodation forms that you are referring.

V/r

Hon. Kevin Turnage MSN Ed. RN, AMB-BC, NHDP-BC, NPD-BC

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From: Greve, Tonie (IHS/PHX) <Tonie.Greve@ihs.gov>
Sent: Thursday, December 15, 2022 7:37 AM
To: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>
Subject: RE: Respiratory Swab Rotation

Good Morning Kevin;

This is discrimination by Management.

Arlene Caucasian (white) Nurse contract working at Indian Health Services.
She is not listed in the Rotation and is a Registered Nurse.

I have a reasonable accommodations and wondering why it is being disregarded.
Your first email stated that as Registered Nurses we can perform duties, but not this contract nurse.

She is the Nurse Triage/Assessment duties but not doing any triaging so why is this Registered Nurse being treated differently?

Respectfully,

Tonie Greve RN CCC Silver & Ocotillo Teams

From: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>
Sent: Wednesday, December 14, 2022 11:08 AM
To: Hill, Elizabeth I (IHS/PHX) <Elizabeth.Hill@ihs.gov>; Greve, Tonie (IHS/PHX) <Tonie.Greve@ihs.gov>; Edison, Bertha (IHS/PHX) <Bertha.Edison@ihs.gov>; Barr, Brenda (IHS/PHX) <Brenda.Barr@ihs.gov>
Cc: Baek, Aimee (IHS/PHX) <Aimee.Baek@ihs.gov>
Subject: Respiratory Swab Rotation

Starting on Monday 12/19/2022 PCMC will begin Respiratory Swabbing with patients in their vehicles. The nurses will cover this and responsibilities will rotate daily as follows:

Tonie Greve
Brenda Barr
Elizabeth Hill
Bertha Edison

Lab draws will still be completed in our Lab Room until the Lab gets their isolation room set-up.

Arlene will remain in triage room.

The process will be as described by Dr. Baek.

Hon. Kevin D. Turnage RN, MSN Ed. AMB-BC, NHDP- BC, NPD-BC
Supervisory Clinical Nurse PCMC and Employee Health , COR II,
Phoenix Indian Medical Center
602-263-1574
Cell: 602-647-9126

"I am not authorized to obligate the government for any work, service or product. Please do not perform any work that will require payment without a valid contract, issued by a government Contracting Officer. Changes to a contract can only be made by the contracting officer. Contractors are reminded to promptly notify the contracting officer of any conduct that may be regarded as a change to the contract in accordance with FAR52-243-7"

Turnage, Kevin D. (IHS/PHX)

From: Greve, Tonie (IHS/PHX)
Sent: Monday, January 23, 2023 8:53 AM
To: Turnage, Kevin D. (IHS/PHX)
Cc: White, Jason (IHS/PHX/PIMC); Lee, Candace S. (IHS/PHX)
Subject: RE: Schedule for today Friday 01/20/2023

Good Morning Kevin;

So your saying that I have to continue with the (9) doctors/providers until when?
This exploitation of me is very stressful, I have done the work of (2) full-time Registered Nurses Clinical Care Coordinators for the whole fiscal year of 2022 during your taking the position.
Prior I was doing this in 2021 under another supervision of management officials.

When will this exploitation of me/Tonie Greve stop?
This stress is very hard to handle day to day, it is causing mental stress, fear.

I keep hearing from management that it will change but you keep the harassment, exploitation of one federal employee and that is a hostile work environment that I have repeatedly ask to stop.
I am including all management officials up the chain so all are aware of this hostile work environment I have had to endure.

<u>I can not continue to perform the work of (2) full-time Clinical Care Coordination Registered Nurses and work as the MSA, and Medical Assistant and the duties of the Clinical Registered Nurses (2).</u>
<u>I will need to work over-time everyday and with this email requesting to work over-time to get the work done of 9 doctors/provider</u>**.**

I am asking for help and to stop the harassment, hostile work environment with the numerous job assignments.
Why am I the only Federal Employee at Primary care medicine clinic mandated to carry the work job assignments?

Respectfully,

Tonie Greve RN CCC

From: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>
Sent: Monday, January 23, 2023 8:41 AM
To: Greve, Tonie (IHS/PHX) <Tonie.Greve@ihs.gov>
Cc: White, Jason (IHS/PHX/PIMC) <Jason.White@ihs.gov>; Hill, Elizabeth I (IHS/PHX) <Elizabeth.Hill@ihs.gov>; Edison, Bertha (IHS/PHX) <Bertha.Edison@ihs.gov>
Subject: RE: Schedule for today Friday 01/20/2023

As far as I am concerned no changes have went into effect because what was sent by Jason different from what I sent out on Jan 4th. Please hold off until I get a hand-off report. I included the other CCC's so everyone knows to hold off.

V/r

Hon. Kevin D. Turnage MSN Ed. RN, AMB-BC, NHDP- BC, NPD-BC
SCN for PCMC Employee Health and Behavioral Health

From: Greve, Tonie (IHS/PHX) <Tonie.Greve@ihs.gov>
Sent: Monday, January 23, 2023 8:22 AM
To: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>
Cc: White, Jason (IHS/PHX/PIMC) <Jason.White@ihs.gov>
Subject: FW: Schedule for today Friday 01/20/2023

Good Morning Team Leaders;

Will this be changed to reflect the changes on the new Panels that are being covered by the RN Clinical Care Coordinators?

There seems to be a lot of confusion this morning by team members.

Please Assist

Respectfully,

Tonie Greve RN CCC

From: Moreno, Michelle (IHS/PHX) <Michelle.Moreno@ihs.gov>
Sent: Friday, January 20, 2023 6:53 AM
To: Begay, Lynette C. (IHS/PHX) <Lynette.Begay@ihs.gov>; PIMC Building 10 <PIMCBuilding10@ihs.gov>; Zah, Brian (IHS/PHX) <Brian.Zah@ihs.gov>; Demers, Larry J. (IHS/PHX) <Larry.Demers@ihs.gov>; Dickerson, Angela M. (IHS/PHX) <Angela.Dickerson@ihs.gov>; Namingha, Denise M (IHS/PHX) <Denise.Namingha@ihs.gov>; Ruiz, Sara L (IHS/PHX) <Sara.Ruiz@ihs.gov>; Moffett, Rebecca L (IHS/PHX) <Rebecca.Moffett@ihs.gov>; Curley, Matthew (IHS/PHX) <Matthew.Curley@ihs.gov>; Namingha, Rochelle (IHS/PHX/PIMC) <Rochelle.Namingha@ihs.gov>; Romero, Mary A. (IHS/PHX) <Mary.Romero@ihs.gov>; Hubbell, Chase (IHS/PHX/PIMC) <Chase.Hubbell@ihs.gov>; PHX/PIMC Inpatient Case Management <PHX_PIMCInpatientCaseManagement@ihs.gov>; Lewis, Breedra (IHS/PHX) <Breedra.Lewis@ihs.gov>; Patten, Michelle (IHS/PHX/PIMC) <michelle.patten@ihs.gov>; Harvey, Evangeline (IHS/PHX/PIMC) <Evangeline.Harvey@ihs.gov>; Sanchez, Aleeza (IHS/PHX/PIMC) <Aleeza.Sanchez@ihs.gov>; White, Jason (IHS/PHX/PIMC) <Jason.White@ihs.gov>; Gomez, Viridiana (IHS/PHX/PIMC) [C] <Viridiana.Gomez@ihs.gov>
Subject: Schedule for today Friday 01/20/2023

Good Morning Everyone,

Here are the changes to the schedule for today, Friday 01/20/2023

Unplanned leave:
Nedra
Brenda J.
Gladys
M. Richardson – Michelle to call patients to reschedule

Scheduled Leave:
M. Sale
Zoe
Colorado
Sharon
Jennifer
Dr. Hassinger (1400-1800)

CME:

Updates:
Ocotillo: S. Jim to be covered by Sharlene, Dr. Bellantoni to be covered by Sharlene, Dr. Baek to be covered by Lynette
Agave Team:
Coral Team: Dr. Pinsonneault to be covered by Erica
Silver Team: Dr. Larosa to be covered by Christine, Dr. Hassinger to be covered by Karyn

Unassigned team
Briggs – Lynette
Samuel – Ojo - Joy

Nurse Advice Line/Nurse Assessment Room
Arlene

Provider of the Day:
Dr. Pinsonneault

Leads:
North: Lori
South: V

PCMC Lab: CLOSED
COVID Testing – Brenda Barr

DCOE ON CALL:
Aaron Cleavenger 602-299-2545

Chief of Medicine:
Dr. Baek

See Jason White if any questions on the schedule.

Thank you,
Michelle Moreno, CMA
602-600-2684

Turnage, Kevin D. (IHS/PHX)

From: Hill, Elizabeth I (IHS/PHX)
Sent: Thursday, July 21, 2022 3:00 PM
To: Turnage, Kevin D. (IHS/PHX)
Subject: Notification of concerns

Importance: High

Kevin,

I wanted to share with you my concerns regarding our skype meeting that occurred on Tuesday, July 19, 2022. I do not feel that my participation in this weekly meeting is productive or necessary for program and/or personal development. The time taken from my daily duties to attend this meeting was not warranted. I feel this time could have been utilized to work on my current workload and projects. We were sent out meeting guidelines and informed verbally at the beginning of the meeting what the goals and objective of the meeting were but we did not discuss anything on the agenda. We instead had to endure 30-45 minutes of one staff member's personal agenda which consisted of the following:

- One person's workload
- One person's viewpoint
- One person's workflow
- One person's questions that were unrelated to the SCN's agenda
- One person's outbursts
- One person's non-participation in meeting
- One person's skype comments
- One person's reluctance to follow guidelines of meeting
- One person's demands for desk audit

I am providing notice that I will not participate in another meeting where this behavior is tolerated. I feel this is a hostile work environment and my contributions are not acknowledged nor will be even heard in this type of environment. This recent meeting made me feel disrespected as a team member and uncomfortable in suggesting any type of improvements d/t sparking a possible rant, personal attack or outburst. The stress this is causing me is not worth me compromising my well-being.

Thank you
Respectfully,

Hd}dehwk#Klo#JQ#
Sulp du|#Frdh#P hglflqh#Frdqlf#
Frdqlfdo#Frdh#Frrugqdwru#
Frudo#Djdyh#) #Gldehwhv#Mrxuqh|#ri#Z hoqhvv#Whdp v#
S=#93508;409:;5#
I=#9350596 0487:#
h0p dle#Hd}dehwk1klo0C lkv1jry

Subject: FW: CCC Provider list
Date: Thursday, January 19, 2023 at 2:20:00 PM Mountain Standard Time
From: White, Jason (IHS/PHX/PIMC)
To: Turnage, Kevin D. (IHS/PHX)
Attachments: image001.png, image002.jpg, image003.jpg, image004.jpg, image005.jpg, image006.jpg, image007.png

---

From: Hill, Elizabeth I (IHS/PHX) <Elizabeth.Hill@ihs.gov>
Sent: Thursday, January 19, 2023 9:30 AM
To: White, Jason (IHS/PHX/PIMC) <Jason.White@ihs.gov>
Cc: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>
Subject: RE: CCC Provider list

Jason,
As a reminder per our verbal conversation on 1/19/23, I would like to respectfully request that future consideration be taken to request that I assume the provider Dr. Sahai as part of my workload distribution. I am aware that this is most likely not a possibility at this time but I wanted to make my request in writing for future consideration. I will also add Kevin as CC just for FYI for future correspondence.

Thank you,
Elizabeth Hill RN, CCC

From: White, Jason (IHS/PHX/PIMC) <Jason.White@ihs.gov>
Sent: Thursday, January 19, 2023 8:44 AM
To: Greve, Tonie (IHS/PHX) <Tonie.Greve@ihs.gov>; Hill, Elizabeth I (IHS/PHX) <Elizabeth.Hill@ihs.gov>; Edison, Bertha (IHS/PHX) <Bertha.Edison@ihs.gov>
Subject: RE: CCC Provider list

Thank you all for your input on this matter.

The majority of you guys have expressed the need to want to keep your teams intact. Trying to keep that in mind, I kept the Silver, Agave, and Coral Teams intact. Ocotillo was the only team that was redistributed.

Tonie		Liz		Bertha	
Provider	Total	Provider	Total	Provider	Total
HASSINGER	862	**WHITE**	1039	**NGO**	986
S. JIM	870	**PINSONNEAULT**	1061	**BLACK**	980
Skantharoopan	779	**PHAN**	964	**TSINGINE**	1037
GOSWAMI	676	**Delgado-Thai**	925	**HUDSON**	943
LA ROSA	783	**RICHARDSON**	923	**BELLANTONI**	274
MILES	846	**Baek**	129	**Sahai**	932
Total	**4816**	**Total**	**5041**	**Total**	**5152**

Please provide your feedback.

Thank you.

From: White, Jason (IHS/PHX/PIMC)
Sent: Tuesday, January 10, 2023 4:09 PM
To: Greve, Tonie (IHS/PHX) <Tonie.Greve@ihs.gov>; Hill, Elizabeth I (IHS/PHX) <Elizabeth.Hill@ihs.gov>; Edison, Bertha (IHS/PHX) <Bertha.Edison@ihs.gov>
Subject: RE: CCC Provider list

I have not seen any comments about this split. I Tonie is out sick this week, but for the rest of you if there isn't any suggestions than this will be the new divide going forward.

Please respond by 1/17 otherwise this will be instituted as is.

Thank you guys!

From: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>
Sent: Wednesday, January 4, 2023 9:32 AM
To: Greve, Tonie (IHS/PHX) <Tonie.Greve@ihs.gov>; Hill, Elizabeth I (IHS/PHX) <Elizabeth.Hill@ihs.gov>; Edison, Bertha (IHS/PHX) <Bertha.Edison@ihs.gov>
Cc: White, Jason (IHS/PHX/PIMC) <Jason.White@ihs.gov>
Subject: CCC Provider list

Below is the new proposed split of providers to CCC. I have tried to average out by number of patients instead of by team, of course this will change as new CCC's come on board. Please review and reply with any concerns/comments at your earliest.

Greve		Edison		Hill	
Black	980	Goswami	676	Baek	129
Hassinger	862	Jim	870	Delgado-Thai	925
Bellantoni	274	Ngo	986	White	1039
Miles	846	Pogo	779	Phan	964
Tsiningine	1037	Richardson	923	Pinsonneault	1061
Hudson	943	Larosa	783	Sahai	932
Total	**4942**	**TOTAL**	**5017**	**TOTAL**	**5050**

Hon. Kevin D. Turnage RN, MSN Ed. AMB-BC, NHDP- BC, NPD-BC
Supervisory Clinical Nurse PCMC and Employee Health , COR II,
Phoenix Indian Medical Center
602-263-1574
Cell: 602-647-9126

"I am not authorized to obligate the government for any work, service or product. Please do not perform any work that will require payment without a valid contract, issued by a government Contracting Officer. Changes to a contract can only be made by the contracting officer. Contractors are reminded to promptly notify the contracting officer of any conduct that may be regarded as a change to the contract in accordance with FAR52-243-7"

Turnage, Kevin D. (IHS/PHX)

From: Edison, Bertha (IHS/PHX)
Sent: Friday, July 22, 2022 11:34 AM
To: Turnage, Kevin D. (IHS/PHX)
Subject: CCC meeting
Attachments: TG.docx

Kevin,

I appreciate everything you do for the clinic. In that regard, the focus of our first CCC meeting was primarily on one's workload and complaints regarding other issues. I didn't think the meeting was beneficial to me, and I don't think it's important to gather every week. Throughout the meeting, I had the impression that nobody wanted to speak up or take part since one of my coworkers kept bringing up unrelated issues. I think the coworker should show more consideration for other people by making an appointment with you to discuss her other issues, rather than interrupting our CC meeting. After a desk audit was ordered, I sat there too afraid to participate in the meeting. I've encountered this person twice since I started working, and I find her acts unacceptable. I've attached a list of the incidents. I do value the effort that everyone does for the company. I wish we could all cooperate in the future rather than compete with one another! I want to work in a hostile-free atmosphere.

Humbly,
Bertha Edison, RNCC

Date: July 22, 2022

To: Kevin Turnage, SCN PCMC

As you kindly know, from February 14, 2022, I have been working as Clinic Care Coordinator for the reputable company Phoenix Indian Medical Center. I've been performing all of my tasks and responsibilities to the best of my ability, and I have a solid history in every way.
Unfortunately, I have been experiencing a difficult situation that has never arisen before in my whole career. I've had a couple of events with Toni Greve, CCC, for some time now because of the way she often harasses and attacks people. My coworker, Toni Greve, tries to intimidate me when she approaches me. At first, I gave her the benefit of the doubt and disregarded it, but then she went beyond what was reasonable. She has expressed regret for her actions, but she still persists. Almost all staff members don't like working with her, she is rude and aggressive and on top of that, she thinks that she is always right.

On March 24, 2022 at approximately 2:00 pm- Toni furiously approached me at my desk and said, "Why are you scheduling patients with the Ocotillo providers, this patient has already seen Dr. Richardson and he should be empaneled with him? The patient should schedule an appointment with Dr. Richardson, according to the PIMC policy and practice for empanelment, which was referenced by the speaker. I replied okay and simply nodded my head as she continued to discuss the procedure and policy. She said, "Liz ought to know the rules by now."
Liz, RNCC, rose up and said, "The rules do not say that, the patient did not establish care with Dr. Richardson, there is no note in the patient chart addressing establishing care, and Dr. Jim was next."
Toni kept talking while furiously walking away since she still didn't agree with it. She came up to me in front of my coworkers, and I was embarrassed by what she said. I went to see Dr. Baek with Liz, and we told her about the incident. My witness was Liz Hill.

On July 12, 2022 at 3:00 pm- Toni, CCC, called me today. She asked me why I was adding new patients to her teams since she claims to have 10 physicians while the rest of us only have 4-5. I informed her that it was only fair for all service providers to add a new patient down the list. But Brenda's job is Brenda's job, not yours, so why are you doing it, she continued. When I told her that I had been given the Peds transition panel/assignment. She inquired as to when I had received this task; I replied that it had been a few months, but she insisted that neither it was my responsibility nor was it part of my job description. She answered, "That's all I need to know" and hung up after I informed her that you and Dr. Baek were responsible for assigning the Peds panel.

I am informing you about the recent encounters that have created a hostile atmosphere in my working environment. I feel like I have been subjected to insults and derogatory remarks by her, Toni Greve.
We must all take proactive measures to be sure employees are aware of, understand and abide by our Workplace Harassment Policy and the U.S. EEOC Sexual Harassment Guidelines.

Sincerely,
Bertha Edison, RNCC